CONSULT FOR AFRICA

Duchess: the 11am prep

Healthcare Risk Management Framework. Presentation, 23 September 2026

Consult for Africa internal. Never send. Never leave in the room.

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hello@consultforafrica.com / +234 913 813 8553 / consultforafrica.com

Lagos and Abuja, Nigeria

0. After the meeting: where this now stands

The meeting happened on 23 September. Three things came out of it and they change the engagement rather than adjust it.

One. The scope moved from clinical to enterprise. The majority of clinical risk management is already in place. What the Hospital wants is an enterprise grade framework covering **operational, financial, cultural and reputational** risk, with clinical integrated into it rather than rebuilt. Everything downstream follows from that sentence.

Two. The fee was too steep, and it now has a reason to fall. This matters. A 45 per cent cut on the same scope would have taught them the rate card is soft and set the reference price for every future piece of Duchess work. A 45 per cent cut because the scope genuinely shrank is a different conversation entirely, and it is the one we are now having. **N35,694,000 to N19,710,000**, 118 resourced days to 55, day rates unchanged. What came out: the deep clinical diagnostic, the case level tracing, the patient safety build, and project coordination, which moves to their team. Say it in exactly that order if it comes up.

Three. Digitisation is live, and it is where the margin comes back. They have digitised most of the work and told us there is room for more. We proposed an intelligent compliance system built on trained models and agents. That is now slide 13 and it is framed deliberately as **framework first, build second, scoped and priced separately**. Do not fold it into the N19.71m. The register has to exist before the system can be shaped around it, and that sequencing is both true and commercially right.

0b. ISO 31000, in enough depth to hold a conversation

Their terms of reference name it, so a board member may well test you on it.

The one fact that matters most: ISO 31000 is a guideline, not a certifiable standard. You cannot be certified or accredited to it, and there is no audit that confers it. Anyone offering to certify a hospital against ISO 31000 is selling something ISO itself does not recognise. Saying this plainly is worth a great deal of credibility in the room, and it also protects them from a vendor who will offer exactly that.

The current edition is ISO 31000:2018, published February 2018, replacing the 2009 version. The revision tightened the language and, most importantly, moved **top management accountability to the centre of the framework**. That is the hook into their governance: the Audit and Risk Sub-committee has to own the tolerance line. We design it, they own it.

Its definition of risk is "the effect of uncertainty on objectives." Deliberately neutral, so risk includes upside as well as threat. That definition is what allows a board to hold opportunity and exposure on one register, and it is the reason an enterprise framework can cover financial and strategic risk without contorting a clinical safety model to fit.

It has three parts and you should be able to name them.

- **Principles.** What good looks like. Risk management should be integrated into the organisation rather than bolted on, structured and comprehensive, customised to the context, inclusive of the people who hold the risk, dynamic as the context changes, built on the best available information, alive to human and cultural factors, and continually improved. All of it in service of one thing, the creation and protection of value.
- **Framework.** The governance conditions that let it work. Leadership and commitment sits at the centre, surrounded by integration, design, implementation, evaluation and improvement. This is the part most hospitals skip, and it is why their register goes stale.
- **Process.** What actually runs. Communication and consultation, then scope, context and criteria, then risk assessment, which is identification, analysis and evaluation, then treatment, then monitoring and

review, then recording and reporting.

Companion standards worth naming if pressed. IEC 31010 is the toolbox of risk assessment techniques, which is where failure mode and effects analysis, bow tie analysis, scenario analysis and the rest actually live. **ISO Guide 73** is the vocabulary. If someone asks which technique we will use, the honest answer is that 31010 offers dozens and the diagnostic decides which fit, rather than picking one now to sound decisive.

Treatment options, in their language. Avoid the activity. Take or increase the risk to pursue an opportunity. Remove the source. Change the likelihood. Change the consequence. Share it, through contract or insurance. Or retain it by informed decision, which is the one boards do by accident and should be doing deliberately.

One more frame worth having, though it is not ISO. The **three lines of defence**: the first line owns and manages risk day to day, the second sets policy and monitors, the third gives independent assurance. In a hundred bed private hospital the third line is almost always the gap, because there is rarely an internal audit function. That is a finding you can predict before you walk in, and it is a natural role for their Audit and Risk Sub-committee to commission rather than for us to occupy.

1. The sixty second read

They issued the terms of reference in March with a 10 April deadline. We responded on 30 March. They are inviting us to present on **23 September**, more than five months after the deadline. That gap is the most important fact in the room and nobody has explained it to us.

Three explanations, and the pitch bends differently under each.

The process simply stalled and has now been reactivated, most likely by a budget cycle or an accreditation decision. Or a first choice was appointed and has not worked out, in which case they are looking for someone who will actually deliver rather than someone who will write. Or something happened, an incident, a claim, a regulatory visit, and risk management has moved from a good idea to an urgent one.

Ask the question in the first three minutes. Warmly, once, and then listen. "Before I take you through anything, help me with one thing. We wrote to you in March. A good deal has happened since. What has changed here, and what is driving the timing now?" Then adapt. If the answer is an incident, everything after that is about speed to a working system. If it is accreditation, everything is about the standard. If it is a failed appointment, everything is about implementation and embedding rather than documents.

2. Where the marks actually are

They published their weighting. Read it before you decide what to spend forty minutes on.

Criterion	Weight	What this means for you
Technical competence and methodology	40%	The single biggest block. Spend most of the presentation here. Method, not credentials.
Relevant experience and track record	30%	Your own operating record plus our hospital engagements. Second most time.
Qualifications of key personnel	20%	Name the team. Do not present roles, present people.
Financial competitiveness and value	10%	The smallest block on the sheet. **Do not lead with price and do not discount to win.**

Seventy per cent of the score is method and track record. Price is one tenth. A competitor who wins this on price has misread the same document you are holding.

3. Five gaps in our own March proposal, and how to close each today

These are ours. Close them before they are raised.

One. We gave them no fee. Their submission requirements asked for a detailed financial proposal including fees, taxes and other costs. Our proposal said a fee schedule would follow the engagement letter. That is a hole and it may well be why this took five months to move. **Walk in with the number.** The one page fee schedule is built and is in the pack.

Two. Our timeline does not match theirs. Their terms of reference say six months. We wrote twelve weeks of core work plus a six month embedding phase, which reads as either thirteen months or as us not having read their document. **Reframe it and get ahead of it:** the system is built and running in twelve weeks, and the remaining months are assurance and embedding, all inside their six month window. We deliver the benefit early, not late. Say it on the timeline slide before anyone asks.

Three. We called her Dr Bukola Ladipo. Their document says Mrs Bukola Ladipo, Human Resources Manager. Check this at the door before you open your mouth. If she is not a doctor, using the title reads as careless, and carelessness is the one thing you cannot afford in a risk management pitch.

Four. The reporting line in their document is the Chief Compliance Officer, not HR. HR ran the procurement. The Chief Compliance Officer is the client. Progress goes to the Board Audit and Risk Sub-committee. **Find out who the Chief Compliance Officer is, whether they are in the room, and speak to them.** If they are not there, ask why and ask to meet them.

Five. Our proposal carries a claim we cannot source. It states that hospitals implementing structured risk management "typically report a 30 to 45 per cent reduction in reportable adverse events within the first year". Do not repeat that number. It is unsourced, and worse, it is the wrong direction. Section 4 turns this into your strongest moment instead.

4. The line that wins the room

This is the thing to say, and it corrects our own proposal while making us look like the only people in the process who have actually run a hospital.

> "If this engagement works, your incident reports will go up in the first year, not down. That is not a failure. A hospital that reports more incidents is a hospital where people have stopped being afraid to report. The number that should fall is harm, and severity, and repeat events of the same type. Any consultant who promises you fewer incident reports in year one is promising you a quieter system, not a safer one, and those are very different things."

Say it slowly. Then the follow through:

> "So we will not measure ourselves on report volume. We will measure on reporting rate against activity, on time from event to report, on the proportion of events with a completed root cause analysis, on repeat events of the same type, and on how many risks on the register actually moved. Those are the indicators we will build into your dashboard."

That single exchange does four things at once. It demonstrates domain command that no generalist firm can fake. It pre-frames the key risk indicator conversation, which is one of their four stated objectives. It protects us from being judged later on a metric that would make good work look like failure. And it quietly removes

the unsourced claim from our March document without ever drawing attention to it.

5. The pitch spine

Forty minutes of content at most, and leave a third of the time for them.

Open, two minutes. Thank them. Ask what has changed since March. Listen. Do not pitch yet.

The problem as we see it, four minutes. Every hospital that commissions this work gets a risk register. Very few have a live one twelve months later. The framework is not the hard part. Any competent firm in this process can write you a policy suite that reads well. The hard part is whether the system is still running after the consultant leaves, and that is decided by three things: whether it was built on evidence your own people produced, whether the reporting line to the board actually functions, and whether somebody inside this hospital owns it.

The counter-intuitive truth, two minutes. Section 4. This is your credibility moment.

Method, eighteen minutes. This is the forty per cent. Four pillars, in this order.

1. **Evidence, not assumption.** Four independent streams, chosen so that no two fail the same way: structured interviews across departments, an anonymous staff safety culture survey, a patient and caregiver experience survey, and documentary analysis of what the hospital already produces. When streams disagree, the disagreement is the finding. On one recent hospital diagnostic the interview evidence alone would have led us to upgrade the revenue cycle, and the documents forced it back down. We would have been wrong without the fourth stream.

2. **The ten case trace.** The differentiator, and nobody else will offer it. Ten real patient journeys, chosen on the morning with their clinical lead present so nothing can be curated, each followed through every link in the chain: presentation, decision to treat, consent, pre-operative assessment, the surgical safety checklist, antibiotic timing, sterilisation load records and instrument traceability, the observation record, escalation and the deteriorating patient, handover, discharge, follow up attendance, the incident log and any complaint. Tracing ten completely beats sampling a hundred partially, because every gap is specific rather than theoretical. You are not telling a board that documentation is inconsistent. You are telling them that in this case, on this date, the sterilisation load record cannot be linked to the patient, so if that load were recalled they could not identify who it touched.

3. **A board line that works.** Their own terms of reference send progress to the Board Audit and Risk Sub-committee. Most consultants have never sat on the receiving end of one of those reports. You have. You are an independent non-executive director of a Lagos specialist hospital and you built its board and committee instruments: the risk summary, the exception report, the escalation thresholds, the registers, the dashboard and the action log. You know the difference between a risk report a board reads and one a board files.

4. **An owner inside the hospital.** Our proposal already commits to identifying and coaching an internal Clinical Governance Champion. Strengthen it in the room: we assess, train and formally certify that person, with a competency check, and we design the role brief so it survives the individual leaving. The champion is the deliverable that makes every other deliverable durable.

Track record, eight minutes. This is the thirty per cent. Section 6.

The team, four minutes. This is the twenty per cent. Named people, not roles. See section 9, it is a decision you must take before you walk in.

Investment, two minutes. This is the ten per cent. Say the number without apology, explain the resourcing behind it, move on. Do not defend it. Do not discount in the room.

Close, two minutes. The three things we need from them, in section 11.

6. The three proof stories

Tell these as stories with a finding in them, not as a client list. Describe every client by sector and scale, never by name, with the single exception in the third.

A paediatric and neonatal facility in Lagos. Commissioned for operational improvement, we ran the four stream diagnostic across ten domains with a formal maturity assessment. The staff safety culture instrument was adapted from a validated international hospital survey with locally written items added. The finding that mattered was procedural, not clinical: a standing instruction in the nursing standard operating procedure required staff to record billing from memory at the end of a shift. That one line explained every data discrepancy the board had been treating as separate problems. **The point for Duchess: risk lives in routines, and you find it in documents and observation, not in interviews.**

A specialist surgical clinic in Lagos. The ten case trace, run end to end, looking at exactly the links a tertiary hospital cares about: sterilisation and load traceability, implant and device traceability and the recall question, the surgical safety checklist, antibiotic timing, perioperative escalation, and the written transfer agreement to a unit with intensive care. **The point for Duchess: this is how we find what your incident log does not contain.**

Havana Specialist Hospital, Lagos. Name this one. You are an independent non-executive director and Consult for Africa was engaged to build the board operating model. Two workstreams: eleven board and committee instruments built with the company secretary, including the risk summary, escalation thresholds, exception reports, registers and the board dashboard, and a governance capability programme delivered across three cohorts, closing with a simulated board meeting containing a planted conflict of interest, a planted reserved matter and a planted variance, plus a per cohort competency check. **The point for Duchess: their deliverables list asks for training materials, session reports and competency assessments. We have done precisely that, this year, in Lagos, at board level.**

Your own record, stated plainly and briefly. Chief Executive of a hospital group in Abuja. Chief Innovation and Strategy Officer at a major international private hospital in Lagos. Founder of a health technology business built across twenty one states and sold to that same hospital group. You have carried the accountability their consultant is being asked to advise on. Say it once, without embellishment, and let it sit.

One caution. Our March proposal says we have supported private hospitals across Lagos, Abuja and Port Harcourt. Be ready to describe those by sector and scale if pressed. If you are asked specifically about Port Harcourt and we do not have one, do not improvise. "Our current engagements are concentrated in Lagos and Abuja" is a complete and honest answer.

7. The number

Anchor: N39.66M standard, one programme concession of N3.97M shown in full, net N35,694,000 for the full six months, on 118 resourced consultant days. With value added tax the total invoice value is N38,371,050. Full workings and the benchmarking behind the number are in `duchess-fee-benchmarking-PRIVATE`. The one page fee schedule breaks it down by phase and grade, which is what a compliance officer and a board sub-committee need to see.

Payment stays exactly as we tabled it in March, because consistency is itself a signal: thirty per cent on engagement confirmation, forty per cent on approval of the framework and policy suite, thirty per cent on completion of handover.

Say the number and stop talking. Do not justify it before anyone questions it.

The concession ladder, private, in this order only

Never concede two things in one sentence, and never concede anything in the room today. Today the answer to price pressure is "let me come back to you with options on shape".

2. **They provide the analyst and project coordinator from their own team.** Saves them N3.36M, reads as N3.36M, and costs us only coordination risk. Ask in return for a named counterpart released at least three days a week.
3. **Shorten the embedding phase from three months to two.** Roughly N5M. Ask in return for a decision this week.
4. **Contract in two commitments.** Phases one and two now, phases three and four on approval of the framework. Costs nothing, reads as shared risk, and it mirrors their own milestone thinking.
5. **Last, and only if it closes today:** a further five per cent, traded for a written reference and the right to use the engagement as an anonymised case study, plus payment within five days of each milestone.

Floor is N26M. Below that the team we described is not the team that turns up, and we would rather lose it.

Free goodwill, spend it late and never early. No mark up on reimbursed travel. Training materials licensed to them in perpetuity. The risk dashboard hosted at no charge for the first twelve months.

Never concede. Partner days, because that is you in the room and it is what they are buying. The independence of the diagnostic. Payment on mobilisation.

8. Questions to ask them

Ask the first one early. Keep three or four in reserve for the discussion.

1. **What has changed since March, and what is driving the timing now?**
2. Who will the consultant report to day to day, and does the Chief Compliance Officer have a direct line to the Board Audit and Risk Sub-committee, or does it route through the chief executive?
3. Has a decision been taken to pursue accreditation, and which standard? Joint Commission, COHSASA, SafeCare or ISO alignment only. The answer changes the framework design materially.
4. **Is there an incident reporting system today, and roughly how many incidents were reported last month?** This is the most diagnostic question on the list. A low number in a busy tertiary hospital is not a good sign, and their answer tells you the true starting point.
5. Is there a risk register, and when was it last reviewed by the board?
6. Who would you see as the internal Clinical Governance Champion, and would that person be released for this?
7. What is the approval path for this decision, and what is the envelope?
8. When do you want to start, and what happens if this slips another quarter?

9. Decisions you must take before 11am

One. The certification question, and it is now a strength rather than a gap. Their terms of reference ask for CPHRM **or equivalent**, marked highly desirable rather than mandatory. The equivalence argument is strong and you should make it without apology: you led quality improvement at Evercare Lekki and championed more than four JCI chapters, you took St Nicholas through COHSASA accreditation, and you implemented SafeCare at Cedarcrest Abuja with Dr Adediwura Okeleye leading on quality. That is three standards across three Nigerian institutions, two of them the standards their own terms of reference name, which is worth more than a certificate held by somebody who has never done it here. Still name the certification itself rather than dodging it: "we hold implementation track record across JCI, SafeCare and hospital quality systems, and if the Council wants a certified holder named on the team we will appoint one."

Two. Who is on the team, by name. Twenty per cent of the score is qualifications of key personnel, and roles do not score. **All five are now named.** You as engagement lead at eleven days. **Matthew Aloba**, Director, as programme lead at twenty nine. **Dr Adediwura Okeleye** on quality and accreditation at thirty six, the most on site of anyone. **Barrister Osahon Omoruyi**, Senior Legal Counsel and Head of Risk at Evercare Hospital Lekki, on regulatory, risk and compliance at fourteen. **Abigail Oladejo** as analyst and project coordinator at twenty eight.

Two things follow from naming Barr. Omoruyi, and both are yours to close before this circulates. He is employed by Evercare, so he needs to agree in writing to being named on a competitor's bid, and to what his fourteen days actually commit him to. And Evercare is another Lagos private hospital, so expect the question. The answer is straightforward and worth rehearsing: he is retained by us as a specialist adviser in his own right, he brings the standard rather than anything belonging to his employer, and everything he sees at Duchess sits under our engagement confidentiality exactly as it would for any member of the team. He has delivered for us before, on the governance programme at Havana.

He also strengthens the certification answer materially. Their bar is CPHRM or equivalent plus ten years in healthcare risk management. A sitting Head of Risk at a major international private hospital is the most direct evidence of that we can put in front of them, and it sits alongside your JCI chapter work and Dr Adediwura's SafeCare implementation.

Three. RESOLVED: you are presenting, and Dr Adediwura Okeleye is on the team. She is named on the deck as quality and accreditation lead at forty nine days, the most on site of anyone, on the strength of leading quality for the SafeCare implementation at Cedarcrest. That answers "who wrote this proposal and are they on the team" in one sentence. Presenting alone also means you carry all four method pillars yourself, so watch the clock: eighteen minutes on method, eight on track record, and do not let the opening question eat the middle of the session.

Four. What you can show live. We can stand up an anonymous staff safety culture survey and a patient experience survey as working web forms with printed QR posters, and a private project page for their document requests and uploads, within days rather than weeks. **If you can show a working instrument on your phone in that room, it is worth more than any slide in the deck.** Decide whether to offer it as a two week proof of capability.

9b. The company profile they also hold

Dr Adediwura sent the **company profile** alongside the proposal. The file is dated 31 March 2026, the day after the proposal went out, so the panel has been reading it for five months. Two things in it help you and three could catch you.

Use these, deliberately

Clinical governance and accreditation is capability three of seven, and it names Joint Commission, COHSASA and SafeCare readiness, governance frameworks, patient safety systems and clinical audit. Say this out loud: this mandate sits inside what the firm already declared it does, rather than being something we are stretching into for a bid.

The five stage execution model, diagnose, design, deploy, deliver, transfer, maps one to one onto the five phases on the timeline slide. Make the link explicitly. It shows the methodology is a firm wide system rather than something written for this tender, and methodology is forty per cent of the score.

"Every engagement starts under NDA." That sentence is already in their hands, which means declining to name clients today reads as policy rather than evasion. Lean on it when you get to track record.

"From 50-bed clinics to 400-bed tertiary hospitals." That is your answer if anyone questions tertiary experience.

"Most consulting firms leave a report. We leave a transformed organisation." Echo it. The deck's spine is the same argument, and a firm that says the same thing in March and September reads as a firm with a settled identity.

Be ready to defend these

The headline numbers. The profile claims 135 plus years of combined senior leadership, twenty plus senior operators, six active engagements, twenty one Nigerian states and five countries. Know which six engagements and which five countries before you walk in. Be aware that other CFA material has carried a 60 plus years figure instead of 135 plus. **Do not say sixty today.**

The Maarova claims, which are the real exposure. Page four of the profile states ninety per cent twelve month retention of assessed hires, and a 0.8 point emotional intelligence improvement at six months. Our own diagnostic on that instrument found the emotional intelligence scale collapsed at the ceiling and no integrity construct present, and our standing rule is that we do not claim predictive validity or normative results until that is fixed. **So do not volunteer the assessment platform at all.** It is not part of this mandate. If the champion assessment comes up, it is "a structured leadership and readiness assessment", which is exactly how the deck words it. If somebody quotes the ninety per cent figure back at you, the honest answer is that those are early cohort numbers on a small base and you would rather show them the instrument than the statistic.

The partner list. Cooked Indoors, Covally, Conexxum, Cureva Health, Health Business Academy Africa and the Doctors Foundation for Care are named as partners and collaborators. If asked, these are ventures and partners in the group rather than hospital clients, and none of them is a reference for this work.

9c. What changed this morning, and how to use it

You chair the Audit and Risk Committee at Havana, and the Nomination, Remuneration and Governance Committee. This is now the strongest single fact in the room and it is on slides 7 and 14. Their terms of reference route progress to a Board Audit and Risk Sub-committee. Most bidders have advised such a committee. You sit in the chair of one. Say it once, plainly, and let them absorb it: you know what makes that committee act because you are the person who decides whether it acts.

Accreditation is now evidenced rather than claimed. More than four JCI chapters championed at Evercare Lekki, COHSASA accreditation at St Nicholas, SafeCare implemented at Cedarcrest Abuja. Three standards, three Nigerian institutions, and their terms of reference name two of the three. This is what answers "have you actually done this" and it carries the thirty per cent.

AI and the platform are now documented, on your instruction. Slide 9 sets out what the tooling does and, more importantly, five controls: no patient identifiable data leaves their environment, every output signed by a named consultant, nothing decided by a model, a log of inputs and outputs for the audit trail, and the report states which analysis was machine assisted. HospitlOS appears as an option they can take or decline. Note this deliberately departs from two standing CFA rules, that we avoid AI framing and never name our platforms in client copy. You directed it, the audience is a compliance officer who will ask, and volunteering the controls before being asked is what turns it from a risk into a credibility win. Lead with the controls, not the capability.

The wider team still needs names. The deck now names you and Dr Adediwura. The remaining three lines are roles. If you can name them in the room, do.

9d. The review: where this submission is still weak

I went back through the whole pack against their terms of reference. Four things are worth knowing before you walk in.

Their submission rules list five things a proposal must contain. We now satisfy three cleanly and two only partly. They asked for an understanding of the terms of reference and methodology, which the deck covers at length. They asked for a detailed financial proposal including taxes, which the fee schedule now does. They asked for a work plan with a Gantt chart, which is now slide 23. The two that remain open are below, and both sit in the twenty per cent and thirty per cent bands.

Gap one, curricula vitae. They asked for detailed CVs of key personnel. We have none attached. The deck names you and Dr Adediwura and commits to full CVs within twenty four hours, which is a reasonable holding position in a presentation but is not what they asked for in the written submission. **Action: get both CVs written against their four stated requirements, the masters degree, CPHRM or equivalent, ten years in healthcare risk management, and tertiary hospital track record, and send them the same day.** Do not wait to be chased.

Gap two, past work with client references. Their fourth requirement is examples of similar past work **with client references**. Our standing position is that we do not name clients, and the deck says so explicitly on slide 16. That is right as a principle and it will read as discipline, but on a scored rubric it is a blank where a competitor will have three referees. **The resolution is to offer named referees on request rather than to publish them.** You can already offer Havana, where you hold a board seat, and the rehabilitation centre in Lekki, which has run in our own advertising, and you can ask one more client for permission this week. Say it in the room: "we do not publish client names, and we will give you two referees on request, with their consent."

The team is no longer the weakest element. All five named, with an unusually strong regulatory line. What remains is the CVs themselves, and none of the five has a consultant profile on the platform carrying a bio or a CV, so all five have to be written rather than exported. **On Matthew:** he is a Director of the firm, staffed on this engagement at principal consultant grade, and the deck says exactly that. Grade and fee agree everywhere, the seniority is disclosed, and there is nothing to explain. Do not volunteer it as a discount argument, because price is ten per cent of their sheet and the point is lost there.

On the wider bench. I searched all sixty eight consultant profiles on the platform and read the bios and CVs of the top candidates rather than trusting the keyword scores. There is no hospital accreditation or risk management specialist on the bench at the level their terms of reference describe. The credible supporting names are Omololuoye Majekodunmi for indicators and monitoring, Francis Mbanefo for the governance and training programme, Omolewa Adedipe for the dashboard workstream, and Daniel Elaigwu Abah for environment of care and biomedical risk, which is a real domain given five theatres and a critical care unit. Only one of the shortlist is Lagos based, which matters for on site days. **If they press for a named CPHRM holder, the honest answer is that we appoint one, and it should be priced into the recruitment rather than promised loosely.**

What is now stronger than it was. The deck opens on their own mission, to achieve clinical excellence and reverse medical tourism, and ties accreditation to it. Slide 3 maps their actual service profile, one hundred beds, five theatres, cardiothoracic, spine, maternity and neonatology, nephrology and advanced diagnostics, to where risk concentrates in each, framed as hypotheses rather than findings. Slide 7 sets out how a risk is identified, scored on the five by five matrix, given a tolerance line by the board, treated and reviewed, which is the methodology depth the forty per cent weighting actually rewards. And slide 24 maps our response to each of their four criteria, so the panel does not have to hunt.

10. Objections, and the answers

"You are not CPHRM certified." Section 9. Name it first.

"Your timeline is twelve weeks, our terms of reference say six months." We build and activate the system in twelve weeks so you get the benefit early, then use the remaining months for embedding and independent assurance against the baseline we set in week two. It is all inside your six months. A framework delivered in month six has never been tested by the time the engagement closes.

"Another firm has quoted significantly less." Your own evaluation criteria weight financial competitiveness at ten per cent and methodology at forty, which tells me you have already worked out where the risk is. The difference between the quotes is the difference between a framework and a framework that is running when we leave. And the cost of getting this wrong is not the fee, it is one claim.

"Have you done this in a tertiary hospital?" I have run one, and I was the strategy and innovation officer of another. Beyond that, section 6.

"Will this framework survive a Joint Commission survey?" It is built to ISO 31000 and the World Health Organisation patient safety frameworks and mapped to the relevant accreditation chapters, so it is designed to be surveyable. We are not an accreditation body and we will not pretend to guarantee an outcome that a surveyor decides. What we will do is give you a readiness scorecard so you know where you stand before you invite anyone.

"How do we know your people will actually be here and not junior staff?" Name the days. Our fee is built on a published day rate per grade and the schedule shows exactly how many days of each grade you are buying. Bill against it.

"Six months from now, what does success look like?" A risk register the board has reviewed at least twice and that has visibly moved. An incident reporting rate that has gone up and a severity profile that has come down. A named certified champion inside the hospital. And a committee that met every time it was scheduled to.

11. The close: what to ask them for

Three things, in this order. Ask, then stop.

1. **A decision date.** Not a decision today. Ask when they will decide and who signs.
2. **A conversation with the Chief Compliance Officer before that date**, if they are not in the room.
3. **Two weeks of proof.** Offer to stand up the staff safety culture survey and the patient experience instrument at no charge before contract, so the board sees real data from their own hospital before they commit. It shortens their decision, it is very hard for a competitor to match, and it puts our instruments inside the building before anyone else gets there.

12. Do not, under any circumstances

- Repeat the 30 to 45 per cent adverse event reduction figure.
- Name another client in that room.
- Discount in the room.
- Use "Dr" for Mrs Ladipo until you have confirmed it.
- Promise a Joint Commission outcome.
- Talk about our own platforms by name. Describe what will be done, not which product does it.
- Leave this document anywhere.